

Stroke / LVO Alert

Standard:

To establish guidelines for declaring a Stroke or Large Vessel Occlusion (LVO) Alert and initiating patient transport to an appropriate receiving facility based on COG transport guideline.

Purpose:

To outline the assessment of a patient currently exhibiting signs and/or symptoms associated with stroke.

Application:

1. Initiate assessment and treatment of suspected stroke patients in accordance with most appropriate clinical guideline(s).
2. Utilize Stroke Screening checklist.
3. Ascertain the last time the patient was seen normal to establish the time of *last known well*.
4. Obtain a blood glucose level according to the blood glucose procedure.
5. Perform the Cincinnati Prehospital Stroke Screen (CPSS).
 - a. All portions of CPSS must be completed. Any abnormality in the screening is positive for stroke.
6. If the has a positive CPSS, then assess for weakness and perform the Visual Disturbance, Aphasia, and Neglect (VAN) Score.
 - a. If any portion of the VAN assessment is positive, then the patient is VAN positive. A positive VAN Score & positive CPSS screening along with weakness is positive for a LVO.
7. Declare the appropriate alert and identify the appropriate destination:

LVO Alert	Stroke Alert
Patient is positive for CPSS, also has weakness, and positive VAN Score	Patient is positive for CPSS but negative VAN Score
Onset of symptoms or last known well time is $\leq$ 23 hours & blood glucose $>$ 50	
Transport to Thrombectomy Capable Stroke Center	Transport to Primary or Thrombectomy Capable Stroke Centers
May transport to Primary Stroke Center (PSC) only if distance to Thrombectomy Capable Stroke Center (TCSC) is $>$ 15 minutes further. For example: Scene to PSC is 5 minutes or 20 minutes to TCSC, then transport to TCSC. Scene to PSC is 5 minutes or 21 minutes to TCSC, then transport to PSC.	
Any patient with stroke-like symptoms with onset $>$ 23 hours should be transported to the closest Stroke Center	
Refer to Transport Guideline and Matrix As Needed	

8. Whenever possible, identify a family member or historian to accompany the patient to the hospital.
9. Tool to aid in capturing patient information for the receiving facility - #FILM
 - a. # - Unit **Number**
 - b. F - **F**amily contact info
 - c. I - Recent **I**nterventions (Surgeries, cardiac cath, etc.)
 - d. L - **L**ast known well
 - e. M - **M**edication (specifically antiplatelet and anti-coagulant)